

Health Intelligence Report

Patient: Marcus Johnson
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NOTICE: This report identifies patterns in health data to support informed conversations with a healthcare provider. It is not a medical diagnosis. Always consult a qualified clinician.

SUMMARY

This patient presents a concerning metabolic profile with suboptimal diabetes control despite medication, alongside early cardiac remodeling and borderline blood pressure control. The constellation of worsening glycemic trends, structural heart changes, and strong family history of premature cardiovascular disease suggests an elevated cardiometabolic risk trajectory that warrants proactive intervention.

PATTERNS IDENTIFIED

Deteriorating Diabetes Control Despite Maximal Metformin Monotherapy

Confidence: Strong pattern

RELATED CONDITIONS

Type 2 Diabetes · Hypertension

SUPPORTING EVIDENCE

- HbA1c plateaued at 7.1% (above target <7%) — Lab Results, March 2026
- Fasting glucose elevated at 126 mg/dL (ref: 70-100 mg/dL) — Lab Results, March 2026
- Home blood glucose trending upward from 118 to 132 mg/dL — Vitals, May 2026
- Currently on metformin 1000mg twice daily (maximal dosing) — Medications, March 2019 onwards

Early Cardiac Remodeling in Context of Metabolic Syndrome and Family History

Confidence: Strong pattern

RELATED CONDITIONS

Hypertension · Type 2 Diabetes · Heart Disease

SUPPORTING EVIDENCE

- Echocardiogram showing mild concentric left ventricular hypertrophy and Grade I diastolic dysfunction — Imaging Studies, April 2026
- Chest X-ray noted borderline cardiomegaly — Imaging Studies, February 2026
- Blood pressure persistently at 138/88 mmHg (above target <130/80 for diabetes) — Vitals, April-May 2026
- Father had myocardial infarction at age 58 (premature CAD) — Family History, July 2019
- Currently on lisinopril 10mg (low-dose ACE inhibitor) — Medications, November 2018 onwards

Weight Stability Despite Suboptimal Metabolic Control

Confidence: Possible pattern

RELATED CONDITIONS

Type 2 Diabetes · Sleep Apnea · Hypertension

SUPPORTING EVIDENCE

- Weight stable at 218 lbs across recent measurements — Vitals, April-May 2026
- HbA1c and glucose trending upward despite no reported weight gain — Lab Results & Vitals, March-May 2026
- Sleep apnea requiring nightly CPAP use — Health History, February 2020 onwards

Preserved Renal Function Despite Diabetes Duration — Monitoring Window

Confidence: Possible pattern

RELATED CONDITIONS

Type 2 Diabetes · Hypertension

SUPPORTING EVIDENCE

- eGFR at 72 mL/min/1.73m² (within normal range, Stage 2 CKD threshold) — Lab Results, March 2026
- Kidney ultrasound showing normal size and echogenicity with no nephropathy signs — Imaging Studies, March 2026
- Diabetes duration of 7 years with currently suboptimal control — Health History & Lab Results, 2019-2026

- On ACE inhibitor (lisinopril) which provides renal protection — Medications, October 2018 onwards

INFORMATION GAPS

- Urine albumin-to-creatinine ratio (UACR) — critical for early diabetic nephropathy detection given 7-year diabetes duration and family history
- Lipid panel breakdown (HDL, triglycerides) — LDL is available but full cardiovascular risk assessment requires complete lipid profile given family history of premature CAD
- Body Mass Index (BMI) calculation — weight is documented but height is needed to assess obesity classification and inform weight management targets
- CPAP compliance data — adherence metrics would help assess whether sleep apnea is adequately controlled, as poor control impacts blood pressure and glucose metabolism
- Previous HbA1c trend data — only single March 2026 value shown; historical trend would clarify if 7.1% represents deterioration or plateau

TALKING POINTS FOR YOUR PROVIDER

- Given the upward glucose trend despite maximal metformin dosing, should we consider adding a GLP-1 agonist or SGLT2 inhibitor, particularly given the cardiovascular and renal protective benefits relevant to your risk profile?
- The combination of structural heart changes, borderline blood pressure control on low-dose lisinopril, and your father's early heart attack suggests we should discuss intensifying blood pressure management and whether additional cardiac risk stratification (such as coronary calcium scoring or stress testing) is warranted.
- Since your weight has been stable but diabetes control is worsening, should we explore whether your current nutrition plan needs adjustment, and discuss the potential metabolic benefits of even modest weight reduction (5-7%) given your sleep apnea and cardiovascular risk factors?
- While your kidney function currently looks reassuring, the eGFR of 72 is approaching the stage where subtle decline could begin. Should we establish a urine albumin-to-creatinine ratio baseline to catch early diabetic kidney disease, especially as we consider medication adjustments?

